Synthetic Policy & Guideline Pack

As of: 2026-10-27 | **Plan year:** 2026-01-01 to 2026-12-31
Notice: Entirely fictional, created for an interview exercise. Not real insurance terms. Companion to `claims_triage_dataset.xlsx`.

1. Plan Summaries

	Bronze HMO	Silver PPO	Gold PPO	Platinum EPO
Plan ID	PLN-BRZ-HMO	PLN-SLV-PPO	PLN-GLD-PPO	PLN-PLT-EPO
Deductible (individual)	$5,000	$2,500	$1,000	$0
Out-of-pocket max (individual)	$8,500	$7,000	$5,000	$3,500
PCP visit copay	$40	$30	$20	$10
Specialist copay	$80	$60	$40	$25
ER copay (waived if admitted)	$500	$350	$250	$150
Coinsurance (member share)	40%	30%	20%	10%
Out-of-network	Emergency only	Covered, 50% member coinsurance	Covered, 40% member coinsurance	Emergency only
Specialist referral required	Yes (from assigned PCP)	No	No	No
Psychotherapy visits per year	12	30	Unlimited	Unlimited

2. Evidence of Coverage (Excerpts)

2.1 Eligibility

A member is covered for a service only if, on the **date of service (DOS)**, the member's coverage was in force: the effective date is on or before the DOS, the termination date (if any) is on or after the DOS, and premium is paid through the DOS. Coverage ends at the end of the termination date shown on the enrollment record. There is no retroactive reinstatement after voluntary disenrollment.

2.2 Order of Cost Sharing

1. **Copay-only services** (office visits, behavioral health visits, ER visit copay): the member pays the copay. The copay is **not** subject to the deductible. Copays count toward the out-of-pocket (OOP) maximum.
2. **All other covered services:** the member pays the remaining deductible first, then coinsurance on the rest of the allowed amount.
3. Once the OOP maximum is reached, the plan pays 100% of allowed amounts for covered in-network services.
4. Cost sharing is always computed on the **allowed amount** (the in-network fee schedule), never on the billed charge.

2.3 Network

In-network providers are paid the allowed amount and may not bill the member beyond cost sharing. HMO and EPO plans do not cover out-of-network non-emergency care. PPO plans cover out-of-network care with higher member coinsurance and the member may be balance billed.

2.4 Emergency Services

Emergency care is judged by the **prudent layperson standard**: symptoms that a reasonable person would believe require immediate attention. The final diagnosis does not decide whether the visit was an emergency. Emergency

services at an **out-of-network** facility are covered at **in-network cost sharing** on every plan, and the member must not be balance billed beyond that. Any difference between billed charge and the plan payment is handled between plan and facility, never charged to the member. All out-of-network emergency claims require human review (rule EMR-01).

2.5 Prior Authorization (PA)

Services marked "PA required" in `Benefit_Rules` need an approved PA covering the DOS before the service. Retroactive PA is allowed only for urgent or emergent situations, and the request must be submitted within **2 business days** of the service. Without an approved PA, the claim is **pended** (not denied) for a PA/clinical-information request. A denial for lack of medical necessity must be made by a licensed clinician.

2.6 Exclusions

Not covered: cosmetic services performed solely to improve appearance, experimental or investigational treatments, and services not medically necessary. See Clinical Policy CP-COSM-001.

2.7 Behavioral Health

Plans cover outpatient psychotherapy up to the annual limit shown in Section 1. Behavioral health visit limits are **quantitative treatment limits** and must comply with mental health parity requirements: they may not be more restrictive than limits applied to comparable medical/surgical benefits. A claim exceeding a behavioral health visit limit **must not be denied automatically**. It goes to Clinical/Compliance review, which checks parity and medical-necessity exceptions (see CP-BH-002).

2.8 Appeals

Every denial notice must state the specific reason in plain language, cite the plan provision or rule, and explain the member's and provider's right to appeal within 180 days.

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3. Claims Adjudication Guidelines (Internal)

3.1 Required Fields

Member ID, billing provider ID, **rendering NPI (professional claims)**, DOS, at least one **valid** diagnosis code, at least one service line with CPT and billed amount. If any are missing or invalid, **pend and return to provider**. Do not infer or "fix" codes. Timely filing: claims must be received within 180 days of DOS.

3.2 Recommended Validation Order

1. Intake and field validation (VAL-01, VAL-02)
2. Eligibility on DOS (ELIG-01)
3. Duplicate check (DUP-01/02)
4. Network status and benefit lookup
5. Prior authorization (PA-01)
6. Exclusions, medical necessity, bundling (EXC-01, MN-01, BUN-01)
7. Provider-pattern and amount flags (EM-01, AMT-01, CHG-01)
8. Pricing and cost-share calculation
9. Decision, explanation, audit log

3.3 Duplicates

A claim is a duplicate if a **paid** claim exists with the same member, billing provider, DOS and CPT code, and the new claim's frequency code is 1 (original). Frequency code 7 means replacement and 8 means void; these adjust the original claim and are not duplicates. A similar claim on a **different DOS** is not a duplicate.

3.4 Bundling

Component tests billed with a comprehensive panel by the same provider on the same DOS are not payable separately. Example: glucose (82947) and creatinine (82565) are components of the comprehensive metabolic panel (80053). Deny only the bundled lines; pay the rest of the claim.

3.5 E/M Level Review

Level-5 office visit (99215) requires documentation of high-complexity medical decision making or at least 40 minutes total time. If a provider's level-5 share is at least 40% **and** at least 3x the peer median, route 99215 claims to Payment Integrity. Down-coding or recoupment is a human decision.

3.6 Decision Authority and Thresholds

Decision	Allowed automatically?	Condition
Auto-approve (pay)	Yes	All validations pass, model confidence >= 0.90, total billed <= $2,500, no risk flags
Auto-deny: eligibility, duplicate, bundled line	Yes	Rule is unambiguous, supporting evidence cited
Auto-deny: clear contractual exclusion	Yes	Total billed <= $10,000 and documentation is consistent with exclusion
Deny for medical necessity	**No**	Always a licensed clinician
Deny for behavioral health benefit limit	**No**	Clinical/Compliance review first
Request information (pend)	Yes	Missing/invalid data, missing PA, missing clinical documents
Escalate to human	Required	Total billed > $10,000; out-of-network emergency; provider pattern flag; confidence < 0.75; conflicting documents

3.7 Explanation and Audit Requirements

Every decision must record: decision, reason code(s), plain-language explanation (member-readable, no jargon), rules fired, data sources and documents consulted, calculated amounts, model confidence, and whether a human was involved. Logs must reference members only by member ID.

3.8 Data Handling

Use only the minimum necessary data. Do not place names, birth dates, addresses or national IDs into prompts or logs. All records in this exercise are synthetic.

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4. Required Decision Output (Suggested Schema)

```
{
  "claim_id": "CLM-2026-0000",
  "decision": "APPROVE | DENY | PEND_REQUEST_INFO | ESCALATE | PARTIAL_PAY",
  "confidence": 0.00,
  "reason_codes": ["R000"],
  "line_decisions": [
    {"line_no": 1, "cpt_code": "00000", "status": "PAID | DENIED | PENDED",
     "allowed_usd": 0.00, "member_responsibility_usd": 0.00, "plan_payment_usd": 0.00, "reason_code": "R000"}
  ],
  "explanation_plain_language": "...",
  "rules_fired": ["ELIG-01"],
  "evidence": ["Members.status", "DOC-C000-00"],
  "route_to": "none | UM nurse | Payment Integrity | Clinical/Compliance | Senior reviewer | Provider outreach",
  "next_actions": ["..."]
}
```

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5. Clinical Policy Excerpts

CP-ORTHO-029: Knee arthroscopic meniscectomy (CPT 29881). Covered when ALL apply: (a) diagnosis of meniscal tear or derangement (e.g., M23.221, S83.211A, S83.241A); (b) MRI or exam findings confirming the tear; (c) mechanical symptoms (locking, catching) OR at least 3 months of failed conservative therapy; (d) approved PA on file. A pain-only diagnosis (e.g., M25.561) does not establish medical necessity.

CP-COSM-001: Cosmetic services. Procedures performed to improve appearance without correcting a functional impairment are excluded. Rhinoplasty (30400) is covered only when documentation shows a functional indication such as nasal airway obstruction with a supporting diagnosis (e.g., J34.2), and PA is approved.

CP-EM-004: E/M documentation. Levels are supported by medical decision making or total time. A single self-limited problem with straightforward decision making supports a low-level visit (99212 or 99213), not 99215.

CP-BH-002: Behavioral health benefit limits. When the annual visit limit is reached, the provider may request additional visits with a clinical justification (e.g., standardized symptom scores, treatment plan). Requests and claims past the limit are reviewed by a clinician and by Compliance for parity before any denial.

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6. Coding Conventions Used in the Dataset

Item	Values
Place of service	10 = Telehealth (patient home), 11 = Office, 22 = Outpatient hospital, 23 = Emergency room, 24 = Ambulatory surgery center, 81 = Independent laboratory
Claim frequency code	1 = Original, 7 = Replacement, 8 = Void
Modifiers	RT = Right side, 95 = Synchronous telehealth
Diagnosis pointer	A = first (primary) diagnosis listed on the claim
Charge outlier flag	Billed > 3x allowed (informational only)

CPT descriptions in the dataset are paraphrased. ICD-10-CM codes are a small illustrative subset. Verify against official code sets for any real use.